جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نسرين عبدالتواب فراج ابوالعلا
المحافظة : القاهره **الحي / المدينة :** منشيه ناصر **العنوان :** ----
رقم البطاقة : 28408230101501 **رقم التليفون :** 01205739157 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 23-08-1984 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نسرين عبدالتواب فراج ابوالعلا
National ID: 28408230101501

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

me	: الأستاذه / نسرين عبد التواب فراج	Request No.	: 13007437
	: 28408230101501	Request Date	: 25/05/2023
	: 1581/23	Verification Date	: 07/06/2023
	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
	: 38 Y	Department	: Out Patient
) Doc.	:	IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

ata: Right breast mass.

Specimen: Tru-cut biopsy, collected at 24/5/2023, 9:57 am.

amination:
grayish white tissue cores up to 1.5cm, totally submitted.
s prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER,
2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

opic: examination:
gical type: Invasive duct carcinoma.
(differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
carcinoma in situ: Focal.
ural invasion: No.
nvasion: No.
onal pathologic findings: Lymphocytic infiltrate.
positive control showed appropriate reactivity):
strong nuclear reaction in 90% of tumor cells (Allred score 8 /8).
trong nuclear reaction in 30% of tumor cells (Allred score 6 /8).
Weak incomplete membrane reaction in >10% of tumor cells (score 1+).
28% nuclear reaction of tumor cells.

sis:
reast mass, Tru-cut biopsy:
ive duct carcinoma G II with focal DCIS.

positive (score 8 /8).

Verified by :
DR.Menna Nabil

Room No. 60 - First Floor Building No. 847, Nile Corniche - After El Qudemah Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name: نسرين عبدالتواب فراج ابو العلا **MR. :** 2405	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

A case of premenopausal right-tissue breast invasive ductal carcinoma (IDC) ER positive, PR positive, HER2 negative, staged T2 N1 (1/11) with upfront breast conserving surgery (BCS). The patient did not receive postoperative radiotherapy due to pregnancy and is currently on adjuvant fulvestrant (FEMARA) and goserelin (ZOLADEX).

The patient is receiving zoledronic acid (ZOMETA) only, after laboratory work-up, at the pharmacy in the daycare unit.

Today the patient was given fulvestrant and a 3-month supply of goserelin; the next dose of goserelin is scheduled for 22/07.

On 22/07 the medical oncology team noted that the patient, a premenopausal pregnancy-associated breast cancer patient diagnosed in the first trimester, underwent right-tissue BCS in August 2023 (T2 N1, 1/11) with IDC, ER 8/8, PR 6/8, HER2-negative. The patient did not receive postoperative radiotherapy and did not attend the scheduled delivery in February 2024. She has been on adjuvant aromatase inhibitor (AI) and goserelin since August 2024, with ongoing fulvestrant therapy.

On 08/09 the patient attended for zoledronic acid and fulvestrant. Laboratory results were accepted but the complete blood count showed a total leukocyte count of 2.19 × 10^9/L with a neutrophil count of 0.58 × 10^9/L, prompting administration of 1 mg G-CSF. A bone scan performed on 06/08/2026 revealed a solitary, intermittent sternal lesion at the manubriosternal junction on the chest bone window.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

الأستاذه / نسرين عبد التواب فراج		Request No.	13007437
28408230101501		Request Date	25/05/2023
1581/23		Verification Date	07/06/2023
Female		Financial Category	اورام دار السلام - نفقة الدولة
38 Y		Department	Out Patient
oc.		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

(score 6 /8).
1+).
e in 28% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

fied by :
Menna Nabil

Room No. 60 - First Floor - Building No. 89 - Nile Corniche - After El Qadeseia Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــحة والسكـان
الهيئة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nesrin Abd Al Tawwab Farraag Abu Al Ela	Date : 25/08/2024
Exam : MAMMOGRAPHY BOTH	Age : 40 Year(s)
Medical ID: 2511/2023	Pat. ID: 40343

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in right side

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C).
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Wafaa Ali

25/08/2024 1:38 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
ادارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nesrin Abd Al Tawwab Farraag Abu Al Ela	Date : 24/01/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 2511/2023	Pat. ID: 40343

BILATERAL BREASTs HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in right breast.

She is pregnant.

Technique

- Bilateral breast ultrasound

Report:

- Right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

- Signature Dr. Nehal Hamdy
- 24/01/2024 11:47 AM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
ادارة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nesrin Abd Al Tawwab Farraag Abu Al Ela	Date : 24/01/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 2511/2023	Pat. ID: 40343

BILATERAL BREASTs HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in right breast.

She is pregnant.

Technique

- Bilateral breast ultrasound

Report:

- Right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

- Signature Dr. Nehal Hamdy
- 24/01/2024 11:47 AM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Nesrin Abd Al Tawwab Farraag Abu Al Ela	Date : 27/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 2511/2023	Pat. ID: 40343

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Status Post right CBS and ALAND, routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ, posterior third architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- **Ultrasound** demonstrated UOQ posterior third, area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses , distortion, asymmetry , or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions .
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-operative sequalae

Left breast (BIRADS 1): Normal study.

Recommendation:-

For annual follow up.

Best Regards,
DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Mammogram
[[DOCTYPE:MAMMOGRAM]]

24, 15:29

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

3- Medical number: 83150946
Old Med : 2511-2023

Female

Patient ID:
Name : نسرين عبدالتواب فراج ابوالعلا

Sex:

Patient Age : 40 Y

0 0

number:

MDT

Family history	Family History	Phone	Phone 01227290819	BMI	Date	REASON OF MDT	REASON OF MDT REMOT
Marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP A	Date of MDT	Date of MDT 17/09/2024
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT: GII WITH FOCAL DCIS	8/8 ER COMMENT:	6/8 PR COMMENT:	0 HER 2 NEU COMMENT:	SISH COMMENT:	25

T	N	M
2	0	

Sonomammography
AXILLARY TAIL SHOWS SPECULATED MASS WITH MICROCALCIFICATION INSIDE
cesm unifocal

PATIENT HISTORY
RT BCS+ALND : IDC GII free resected margins LNs 1/11 pT2 PN1
LAST MDT:pt counseled her obstetrician , she is =/- 13 weeks pregnant (ended 1st trimester)
after pt counseling she is refusing cth with pregnancy
pt had labour 4 months ago
on AI - ZOLADEX
new CT 8/2024 revealed liver & bone mets

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
bone scan
continue same treatment AI -ZOLADEX (as first line metastatic)

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]